

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Office Visit - Jul 01, 2026

with Patrick Brennan, MD, PhD at Brigham and Women's Allergy and Immunology Clinic



Notes from Care Team



Progress Notes by Patrick Brennan, MD, PhD at 7/1/2026 1:40 PM

Brigham & Women's Hospital Allergy & Immunology Clinic

7/1/2026

Dear Dr. Clark,

We had the pleasure of seeing Mr. May in the Brigham and Women's Hospital Allergy and Immunology Clinic for evaluation of chronic sinusitis without nasal polyposis. He was recently seen by and referred to me by Dr. Stella Lee in ENT, and has previously been seen by MEEI. He and his ENT team has concerns about allergy, biofilm, immunodeficiency, and potential structural issues with his nose. His full recent ENT reports are not available at the time of writing this note.

Sinusitis, Seasonal and Perennial Allergic Rhinitis:

- Sneezing: No
- Nasal congestion / runny nose: Yes - congestion and blockage
- Itchy eyes: No
- Sinus infections: Yes, chronic sinus infection with monthly flares.
- +purulent mucus - he showed me thick, yellow, elastic-looking mucus
- +green mucus vomited up

Symptoms have been present for many years, worse over the past 2 years. His symptoms seemed to start after he broke his nose playing hockey, and

he did not have longstanding issues with allergy or sinusitis prior to this time.

He underwent a total of 6 surgeries at MEEI, last 2013.

-none of his surgeries have provided significant benefit with his sinusitis symptoms

He has been seen by ID at Lahey health clinic 03/26/26, and was placed on weekly azithromycin. He did not find any benefit from this suppressive antibiotic regimen.

Dr. Lee sent a deep sinus culture yesterday, but the results are not yet available.

-Suspected Triggers:

Pollens: No, Seasons: N/A

Dust mite: Yes, possibly he has sensitization to this allergen due to morning symptoms. He had dust mite allergy diagnosed as a child, but does not consider himself particularly allergic to dust as an adult.

Mold: Yes, he has mold in his apartment, but not clearly worse in his apartment. He has found mold in his basement and in one bathroom.

Cat/Dog: No

Medications tried:

Antihistamines: No

Nasal steroids: Yes
-minor benefit

-budesonide with xylitol rinses were recommended by Dr. Lee, but he is not currently taking this therapy.

Astelin: No

Atrovent: No

Nasal saline rinses: Yes

Controlled overall? No

Antibody-mediated Infection Screen:

Sinusitis: many episodes per year.

-suspected allergies? possible

-resolved with antibiotics? no

-ENT evaluation? Yes x 2

Pneumonia: None

Otitis media: Yes, multiple per year, not always associated with sinusitis.

Meningitis: None

Sepsis: None

Other infections:

-Cellulitis: None

-Giardia: None

+oral ulcers recently

06/29/26, recent ENT (portions), Dr. Rahmati - seen in the MEEI ER for ear symptoms

Assessment & Plan 🗣️

1. Sensation of fullness in both ears

Persistent bilateral ear fullness with left side worse. Left ear tube visualized in place and dry, no evidence of infection.

- Discussed referral to otologist for further evaluation
- Refer to otologist for comprehensive evaluation

2. Otalgia of both ears

Reports bilateral ear pain, ongoing despite previous interventions. No acute infection observed on exam.

- Refer to otologist for further assessment

3. Mouth ulceration

Multiple oral ulcers present since 06/26/2026-06/27/2026. Lesions observed on tongue, hard palate, and cheek.

- Provided education on use of steroid paste for oral ulcers and expected healing time
- Prescribe steroid paste for topical application twice daily for 1 week

4. Chronic sinusitis, unspecified location

Chronic sinus congestion and mucus production for >1 year. Previous budesonide and saline rinses not effective. Sinus CT from 08/2025 showed mild paranasal sinus disease.

- Discussed ongoing evaluation with new sinus specialist and importance of follow-up
- Reviewed prior sinus CT and MRI brain
- Continue evaluation with new sinus specialist (Dr. Lee)

5. Possible immune deficiency or autoimmune disorder

Recurrent oral ulcers and chronic ENT symptoms raise concern for immune dysfunction. No prior immune workup documented in this system.

- Discussed need for immune evaluation and possible autoimmune etiology
- Refer to allergy and immunology for immune workup

6. Gastrointestinal symptoms

Reports irregular stools and abdominal noises. No GI evaluation completed yet.

- Refer to gastroenterology for further evaluation

Procedure: Nasal Endoscopy (CPT 31231)

Surgeon: RAHMATULLAH RAHMATI, MD

Indication: Chronic sinonasal symptoms unexplained by anterior rhinoscopy
Informed Consent: The details of the procedure including risks, benefits, and alternatives were discussed with the patient and verbal consent obtained to proceed.

Procedure: Topical spray consisting of 1% oxymetazoline and 1% lidocaine was sprayed into the nasal cavity bilaterally. A flexible nasopharyngoscope was then used to visualize each nasal cavity (septum, turbinates, middle and superior meatus, sphenoethmoidal recess), and nasopharynx. The oropharynx, larynx, hypopharynx were also visualized.

Findings: The nasal septum was deviated to the left. Inferior turbinates were hypertrophic. Left middle meatus: no edema, no purulence or polyposis. Left superior meatus: clear. Left sphenoethmoidal recess: clear. Right middle meatus: no edema, no purulence or polyposis. Right superior meatus: clear. Right sphenoethmoidal recess: clear. Nasopharynx and Eustachian tube orifices: No masses, lesions, or obstruction.

Attending: Rahmatullah Rahmati, MD

CT Face (Sinus) Without Contrast

Performed: 8/11/2025 at 11:53 AM

Accession Number: E60161009

Reason For Exam

* Sinusitis, chronic or recurrent

Exam Images

[Show images for CT Face](#)
All Patient Images

[Show images for May, Daniel S](#)

Impression

Scattered mild paranasal sinus disease, as described. Aerated secretions in the right maxillary sinus may reflect an acute component in the appropriate clinical setting.

Narrative

CT FACE (SINUS) WITHOUT CONTRAST

Referring clinician's provided indication for this examination in Epic: * Sinusitis, chronic or recurrent

TECHNIQUE: Multidetector-row CT of the sinuses was performed without intravenous contrast using tailored dose modulation techniques. Images were reconstructed in the axial, coronal, and sagittal planes.

COMPARISON: CT HEAD WITHOUT CONTRAST 2021-Mar-19

FINDINGS:

Frontal sinuses and frontoethmoidal junctions: Mild mucosal thickening.

Anterior and posterior ethmoid air cells: Mild mucosal thickening, left greater than right.

Maxillary sinuses and infundibula: Mild circumferential mucosal thickening, right greater than left, with aerated secretions in the right maxillary sinus. Opacification of the left infundibulum and right infundibular ostium.

Sphenoid sinuses and sphenoid ethmoidal recesses: Mild mucosal thickening.

Nasal cavity: Similar subtle leftward nasal septal deviation.

Imaged maxillary teeth: No periapical lucencies. Again noted is a cerclage at the anterior nasal spine.

Mastoid air cells and middle ear cavities: Partial opacification of the left mastoid air cells. The right mastoid air cells are well-aerated.

Temporomandibular joints: No significant degenerative remodeling.

Brain: Images of the brain parenchyma are not of diagnostic quality for the soft tissues. No focal abnormality is visible with this technique.

Orbits and globes: No abnormality.

Other: Right palatine tonsilloliths.

Patient Active Problem List

Diagnosis

- Open angle with borderline findings and high glaucoma risk in both eyes
- Vision changes

No Known Allergies

Current Outpatient Medications

Medication	Sig	Dispense	Refill
• budesonide (PULMICORT) 0.5 mg/2 mL nebulizer solution	Add one vial to 240ML neilmed sinus rinse and rinse nasally daily	60 mL	11
• budesonide (PULMICORT) 0.5 mg/2 mL nebulizer solution	Take 2 mL (0.5 mg total) by nebulization 2 (two) times a day. Mix 1 vial in 8 oz distilled water and irrigate 1/2 bottle each nostril twice daily	120 mL	6
• budesonide (PULMICORT) 0.5 mg/2 mL nebulizer solution	Take 2 mL (0.5 mg total) by nebulization 2 (two) times a day. Mix 1 vial in 8 oz distilled water and irrigate 1/2 bottle	120 mL	6

	each nostril twice daily		
• clonazepam (KLOPIN) 1 MG tablet	Take 1 mg by mouth 2 (two) times a day as needed for anxiety. (Patient not taking: Reported on 3/20/2026)		
• fluticasone propionate (FLONASE) 50 mcg/actuation nasal spray	1 spray by Nasal route daily. (Patient not taking: Reported on 3/20/2026)	30 mL	0
• naproxen (NAPROSYN) 500 MG tablet	Take 1 tablet (500 mg total) by mouth 2 (two) times a day with meals for 15 days. (Patient not taking: Reported on 3/20/2026)	30 tablet	0
• oxyCODONE 5 MG immediate release tablet	Take 1 tablet (5 mg total) by mouth every 6 (six) hours as needed. (Patient not taking: Reported on 3/20/2026)	15 tablet	0
• triamcinolone (ORALONE) 0.1 % paste	Place onto teeth 2 (two) times a day.	5 g	12

No current facility-administered medications for this visit.

Family History:

No sinus issues, immune issues, or immunodeficiency.

Social History:

Works in real estate. He has limited outdoor activity due to his sinus issues.

Review of Systems:

On ROS, Mr. May reported fevers, chills, weight change, possible enlarged lymph nodes, itchy watery eyes, blurry vision, ear popping, difficulty hearing, postnasal drip, sinus pain and pressure, palpitations, abnormal heart rhythm, shortness of breath, nausea vomiting, diarrhea, painful joints, back pain, dry skin, sensitive skin, rashes, frequent urination, excess thirst, fainting spells, dizziness, increased stress.

Otherwise a complete ROS was negative in detail except as noted above. Please see the patient's intake questionnaire for more details.

Physical Exam:

BP (!) 135/93 | Pulse 95 | Ht 188 cm (6' 2") Comment: Per PT | Wt 80.8 kg (178 lb 1.6 oz) | SpO2 99% | BMI 22.87 kg/m²

Gen: Well-appearing, speaking comfortably in full sentences

HEENT: Clear oropharynx, no exudates, lesions, or thrush. Nares have minimal edema bilaterally, no active discharge, no polyps visible anteriorly. Conjunctivae non-injected, no discharge, no chronic changes to mucosa. Trachea midline. No masses or lymphadenopathy appreciated. Tympanic membranes are normal in appearance bilaterally, as are external auditory canals.

Lungs: CTA bilaterally, no wheeze, rales, or rhonchi. Good air movement.

CVR: RRR, no m/g/r.

Ext: Warm, well-perfused, no cyanosis or clubbing.

Skin: No rash on exposed skin. warm and dry.

Other: A&Ox3, pleasant demeanor.

Assessment and Plan:

Mr. May presents with episodes of recurrent sinusitis, having undergone multiple sinus surgeries both for trauma and recurrent sinusitis subsequently. He does not have strong symptoms consistent with allergic rhinitis, but he could still have a perennial allergic sensitizer contributing to his symptoms. Given the onset of his symptoms after a traumatic event, and the lack of pneumonias, humoral immunodeficiency is unlikely, but is also important to consider. I agree with the patient that he could have an issue with bacterial colonization leading to his symptoms, including the possibility of biofilms, but I do think this is unlikely to be immune mediated.

Sinusitis:**Possible allergic trigger:**

To better define the patient's environmental sensitizations, I recommend allergy testing. We will test a panel of common seasonal and perennial allergens through blood testing. The goals of this testing are to identify sensitizations for avoidance or reduction, to plan seasonal medication use, and for consideration of allergy immunotherapy.

-I will follow up with the patient via the patient gateway

Nasal saline rinses were recommended today. For large volume nasal saline rinses, either a Neti Pot or NeilMed rinse device was recommended. The patient was counseled on the use of sterile, distilled, or previously-boiled water for nasal saline rinses since tap water can rarely harbor infectious organisms.

-I agree with budesonide rinses with xylitol as recommended by Dr. Lee.

-Once he is established with his budesonide rinses, he could consider hypertonic saline rinses once per week. This message leads to nasal irritation, and thinning of existing mucus through water shifts, which can sometimes help patient's clear thick or stuck mucus.

Immune workup:

As outlined above, I recommend that we assess his humoral and cellular immune system through blood based testing. The following tests were ordered:

CBC and Differential

Routine, Clinic Collect, If deemed necessary based upon the results of the automated differential, a manual differential may be performed , Resulting Agency - MGB LAB

IgG Subclasses

Routine, Clinic Collect, Resulting Agency - MGB LAB

Immunoglobulins IgG, IgA, IgM

Routine, Clinic Collect, Resulting Agency - MGB LAB

T-Cell/B-cell Immunodeficiency Panel, Flow Cytometry

Routine, Clinic Collect, Specimen Types - Blood;, Specimen Sources - Blood;, Resulting Agency - MGB LAB

Tetanus Toxoid Antibody, IgG

Routine, Clinic Collect, Resulting Agency - MGB LAB

I will let him know the results of this testing by the Patient Gateway.

Other adjunct sinusitis treatments:

I recommended a trial of an adjunct nasal spray that can be used intermittently on an as-needed basis, or regularly, and can be used safely in combination with nasal steroid sprays and oral antihistamines:

-AzelaStine (Astelin or Astepro) nasal spray is available over the counter and can provide relief of nasal congestion or runny nose. Astelin can have a bitter flavor. This medication works on the day it is taken and requires no build up. Rarely, this medication can lead to mild sedation. This medication was prescribed today. If the over-the-counter version (Astepro) is cheaper, the patient is welcome to use this instead of the prescription version.

- This medication is slightly drying, and if he has worsening his symptoms when starting this medication, he can stop it. This medication is not useful for preventing sinus infections or clearing sinus infections, but can help with the mucus ecosystem in some patients.

Oral ulcers:

There are no immunodeficiencies consistent with the patient's symptoms that I know of that are consistent with oral ulcers. These are also relatively recent.

-I recommend that he follow-up dental medicine regarding his mouth ulcers

Follow up in 3 months or sooner as needed.

Thanks for allowing us to participate in Daniel S May's care. Please let us know if you have any questions.

Patrick Brennan, M.D., Ph.D.
Brigham and Women's Hospital Allergy and Immunology

I personally spent a total of 45 minutes on care for this patient on the date of the encounter. This includes face-to-face time during the visit as well as non face-to-face time spent on chart review, documentation, and care coordination. Portions of this note were completed with voice recognition software and transcription errors occasionally persist despite proofreading - please don't hesitate to contact me for clarification.

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